

Obstetrics
O-04 Childbirth Complications BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Identify the signs of shock
- Determine stage (trimester) of pregnancy (If pregnant)
- Determine the degree of physiologic distress, estimate amount of blood loss
- Assure an advanced life support response

Breech Presentation

Presentation of buttocks or feet.

1. Ensure a patent airway (suction as necessary)
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD;
4. Allow delivery to proceed passively until the baby's waist appears;
5. Rotate baby to face-down position (do NOT pull);
6. If the head does not readily deliver in 4 – 6 minutes, insert a gloved finger into the vagina to create an air passage for the infant;
7. Monitor vital signs frequently;
8. Assist advanced life support personnel with patient packaging and movement to ambulance.

Prolapsed Cord

Cord presents first and is compressed during delivery compromising infant circulation.

1. Ensure a patent airway (suction as necessary)
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD;
4. Insert gloved hand into vagina and gently push presenting part off of the cord. Do NOT attempt to reposition the cord. Cover cord with saline soaked gauze;
5. Place mother in Trendelenberg position with hips elevated;
6. Monitor vital signs frequently;
7. Assist advanced life support personnel with patient packaging and movement to ambulance.

Obstetrics

O-04 Childbirth Complications

PRIORITIES ➤ ABCs ➤ Maintain airway, provide oxygen and ventilatory support PRN ➤ Early Transport. ➤ EARLY CONTACT OF RECEIVING HOSPITAL	Final JULY 2008
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NORMAL PRESENTATION	BREECH PRESENTATION	PROLAPSED CORD
➤ OXYGEN – high flow ➤ Reassure mother, instruct during delivery; ➤ IV access if time allows; ➤ As head is delivered, gently suction baby’s mouth and nose, keeping the head dependent. If cord is wrapped around neck and can’t be slipped over the infant’s head, double clamp and cut between clamps; ➤ Allow delivery, dry baby and keep warm, placing baby on mother’s abdomen or chest; ➤ Allow cord to stop pulsating, then clamp and cut 6 – 8 inches from baby; ➤ Assess APGAR score at 1 and 5 min (Protocol P-2 for Neonatal Resuscitation) ➤ Allow delivery of the placenta; save and bring to hospital.	➤ OXYGEN – high flow; ➤ IV Access IF TIME ALLOWS. ➤ CONTACT BASE PHYSICIAN ➤ Allow delivery to proceed passively until the baby’s waist appears; ➤ Rotate baby to face down position (do NOT pull); ➤ If the head does not readily deliver in 4 – 6 minutes, insert a gloved hand into the vagina to create an air passage for the infant; ➤ If the head delivers, proceed as with normal presentation;	➤ OXYGEN – high flow; ➤ IV Access, IF TIME ALLOWS; ➤ CONTACT BASE PHYSICIAN. ➤ Insert gloved hand into vagina and gently push presenting part off the cord; ➤ Place mother in knee-chest position. If unable, place pillows under hips to elevate them; ➤ Early transport;

DISRUPTED COMMUNICATIONS

In the event that a Solano County EMT-P is UNABLE to make physician contact for orders, the paramedic MAY NOT utilize those areas of the protocol needing physician direction and MUST TRANSPORT IMMEDIATELY TO THE CLOSEST FACILITY.